

URIPAX® F. C. TABLET 200MG

Ingredient(s):

Each tablet contains:

Flavoxate Hydrochloride 200mg

Pharmacology:

Pharmacodynamics:

1. Flavoxate hydrochloride (and its main metabolite methyl flavone carboxylic acid, MFCA) is an antispasmodic selective to the urinary tract. In animal and human studies, flavoxate hydrochloride has been shown to have a direct antispasmodic action on smooth muscle fibres.
2. The mechanism of action involves intracellular cyclic AMP accumulation and calcium blocking activity. It inhibits bladder contractions induced by various agonists or by electrical stimulation and inhibits the frequency of bladder voiding contractions. It increases bladder volume capacity, reduces the threshold and micturition pressure.
3. In addition, animal studies have shown flavoxate hydrochloride to have analgesic and local anaesthetic properties.
4. Flavoxate does not significantly affect cardiac or respiratory functions.

Pharmacokinetics:

1. Oral studies in man have indicated that flavoxate is readily absorbed from the intestine and converted, to a large extent, almost immediately to MFCA.
2. Following single oral dosing to volunteers of 200 mg and 400 mg flavoxate, almost no free flavoxate was detected in the plasma. The peak level of MFCA was attained at 30-60 mins after the 200 mg dose and at around two hours following the 400 mg dose. The AUC for the 400 mg dose was approximately twice as large as the AUC for the 200 mg dose. About 50% of the dose was excreted as MFCA within 12 hours; most being excreted within the first 6 hours.
3. After repeated oral dosing (200 mg, TDS, 7 days) the cumulative excretion of metabolites stabilised at 60% of the dose on the third day remaining almost unchanged after one week.

Indication(s):

Symptomatic relief of dysuria, vesical suprapubic pain, nocturia, frequency and incontinence associated with cystitis, prostatitis, urethritis and urethrorrigoitis. Relief of vesico-urethral spasm due to catheterization, cystoscopy or indwelling catheters, and sequelae of surgical intervention of the lower urinary tract.

Contraindication(s):

Flavoxate is contraindicated in patients who have any of the following obstructive conditions: pyloric or duodenal obstruction, obstructive intestinal lesions or ileus, achalasia, gastrointestinal haemorrhage and obstructive uropathies of the lower urinary tract. Hypersensitive to flavoxate hydrochloride.

Dosage and Administration:

Adults: One tablet three times a day.

Route of Administration: Oral

Warnings and Precautions:

1. Flavoxate hydrochloride should be given cautiously in patients with suspected glaucoma.
2. Where evidence of urinary tract infection is present, appropriate anti-infective therapy should be instituted concomitantly.
3. Effects on Ability to Drive and Use Machines: Patients should be informed that if drowsiness and blurred vision occurs, they should not operate a motor vehicle or machinery or participate in activities where alertness is required.

Side Effect(s) / Adverse Reaction(s):

The following adverse reactions have been observed, but there are not enough data to support an estimate of their frequency:

Blood and lymphatic disorders: Eosinophilia, leukopenia.

Immune system disorders: Hypersensitivity reactions.

Nervous system disorders: Drowsiness, dizziness, headache, mental confusion (especially in the elderly), nervousness, vertigo.

Eye disorders: Blurred vision, disturbances in eye accommodation, increased ocular tension.

Cardiac disorders: Palpitations, tachycardia.

Gastrointestinal disorders: Diarrhoea, dry mouth, dyspepsia, dysphagia, nausea, vomiting.

Skin and subcutaneous tissue disorders: Angioedema, urticaria, erythema, rash, pruritis

Renal and urinary disorders: Dysuria.

General disorders: Fatigue, hyperpyrexia.

Interactions with Other Medicaments

1. Anticholinergic or medications with anticholinergic effect:
Concurrent use may intensify the anticholinergic effects of flavoxate.
2. Central nervous system depression-producing medications:
Concurrent use may potentiate the CNS depressant effects of either these medications or flavoxate.

Pregnancy and Lactation:

Since there is no evidence of the drug's safety in human pregnancy, nor any evidence from animal work that is free from hazard, flavoxate should be avoided in pregnancy unless there is no safer alternative. It is not known whether flavoxate is excreted in human milk. Because many drugs are excreted in human milk, caution should be exercised when flavoxate is administered during breast-feeding.

Symptoms and Treatment for Overdosage, and Antidote(s):

Although no such experience has been reported to date, it is considered likely that the following symptoms may be observed: nausea, vomiting, flushing, dilated pupils, dry mouth and tongue, hot dry skin, fever, sinus tachycardia, hypertension, ataxia, nystagmus, drowsiness, delirium, agitation and visual hallucinations. Uncommonly, systemic features of anticholinergics toxicity include myoclonic jerking, coma, convulsions, cardiac conduction abnormalities and dysrhythmias, cardiovascular collapse, paralytic ileus, urinary retention.

Consider activated charcoal if a patient presents within one hour of ingestion of a potentially toxic amount. Alternatively, the role of gastric lavage should be considered in adults presenting within 1 hour of a potentially life-threatening overdose.

Treatment is supportive and observation for 6 hours after ingestion, without other specific treatment, will be sufficient for the majority of patients.

Shelf-Life:

3 years from the date of manufacture.

Storage Condition(s):

Store at temperature below 30°C. Protect from light and moisture.

Product Description:

A slight-yellow film coated round tablet.

Dosage Forms and Packaging Available:

Tablets in blister packing of 10's x 10, 10's x 50 and 10's x 100.



Manufacturer and Product Registration Holder:
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